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Corruption and the emergence of a healthcare 'marketplace' in Nigeria's primary health centres: reframing drivers and solutions

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ABSTRACT

Corruption in the health systems of low- and middle-income countries (LMICs) is fuelled by many factors, including health system failures, practical norms, informal rules, and social networks. Researchers have historically scrutinized these factors in isolation, often failing to grasp the intricate interconnections and the combined manner in which they manifest. In this ethnographic study, we present evidence from Nigeria's healthcare system, where these factors converge, fundamentally reshaping the structure and operation of the healthcare system away from those set out in policy. Our study entailed three months of simultaneous participant observation at six Primary Health Centres in Enugu, Nigeria by four researchers. Following this, we conducted in-depth interviews with healthcare workers and their managers, and focus group discussions with service users. Our findings show that informal payments and rent-seeking, are widespread. Failures to provide basic infrastructure, staff and commodities means that healthcare providers have to use informal means to raise money in order to address these deficiencies. These practices are deeply embedded in reciprocal obligations among staff members, coordinated by groups who collaborate and engage in sustained interactions. Over time, their shared norms and networks are underpinned by informal agreements when and how to charge patients and allocating collected resources. Under this situation, healthcare facilities metamorphose from the ideal of primary health facilities as a setting in which care is provided according to need, into a 'marketplace,' where access to care and health commodities is profoundly shaped by economic imperatives and intricate social processes. The findings of this study suggest that any single "silver bullet" approach, such as solely focusing on norms, and health system failures, is likely to have minimal impact. Instead, policy makers should seek innovative ways through which more equitable access to care can be achieved in the current context.

1. Introduction

Primary Healthcare facilities are designed to make basic minimum health services accessible to everyone (Behera et al., 2022; Otovwe and Elizabeth, 2017). They are expected to be the most inclusive, equitable, cost-effective, and efficient approach to enhancing the health status of the population (World Health Organization, 2023). Strengthening primary health care is seen as a crucial step in enhancing the performance of health systems, subsequently contributing to the achievement of Universal Health Coverage (UHC) (Van Weel and Kidd, 2018).

Primary Health Centres (PHCs) in sub-Saharan Africa frequently suffer from underfunding and inadequate oversight (Nyikuri et al., 2015). This hampers their ability to deliver essential healthcare services effectively. This is the case in Nigeria, the focus of this paper, where PHCs are very poorly funded and supervised, leaving health providers with a lack of resources and the discretion to determine how health centres are run (Ogbozor et al., 2022).

Most PHCs in Nigeria deliver basic services including antenatal and postnatal care, immunisation, family planning, and screenings for malaria, hypertension, and diabetes (Makanjuola and van Vuuren, 2021).

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There are multiple policies and interventions, including funding designed to improve the quality and equitable access to healthcare services through PHCs (see Fig. 1). Contraceptives and other family planning supplies are expected to be provided by the central government, delivered to PHCs by the state governments and then provided free of charge to patients (USAID, 2015). The Basic Health Care Provision Funds (BHCPF), was established to provide health insurance to the poorest Nigerians with the aim of cutting out-of-pocket (OOP) payments by 30 % and enhancing financial risk protection (National Primary Care Development Agency et al., 2020). The Global Alliance for Vaccines Initiative (GAVI) makes PCV, IPV and Pentavalent vaccines, and other non-vaccine operations available at PHCs, with funding expected to eliminate the need for patients to make payments (Obi-Jeff et al., 2021). Corruption and failures in the delivery of PHC in the Nigerian health sector undermines the effectiveness of these investments and challenges attempts for transformative change (Akwataghibe et al., 2013; Odii et al., 2022; Onwujekwe et al., 2019).

(From Uzochukwu et al., 2018)

Recent research evidence demonstrates how corruption is linked to the widespread use of informal rules and health systems failures in Nigeria (Agwu et al., 2024; Narula, 2020) but these are not the only two factors that perpetuate corruption. It is also sustained through social norms and social relations. Corruption endures when a majority of individuals perceive that corrupt behaviour is tolerated and anticipated by their peers (Baez Camargo and Koechlin, 2018). In contexts where there are divergences between official norms and practices, practical norms prescribe socially acceptable or permitted behaviours and govern the actions of public actors (De Herdt & Olivier De Sardan, 2015). These shape resource allocation and treatment decisions, influencing disparities in access, timing, and quality of delivery (Olivier De Sardan et al., 2017). Informal networks, underpinned by long-term professional and personal relationships among health providers wield significant influence due to their association with norms that emphasize the need to exchange favours and reciprocate gifts (Camargo and Passas, 2017). They become an effective mechanism for pooling scarce resources and providing access to goods and services in the context of resource scarcity (Baez Camargo and Koechlin, 2018). They can be used as a coping strategy, which means that individuals may turn to their informal networks for support or assistance in dealing with challenges at work, thus having both positive and negative effects on service delivery (Odii et al., 2022). Social networks contribute to the proliferation of petty corruption between service seekers and service providers in East African health sector (Stahl and Kassa, 2018).

Despite these multiple factors that shape and reinforce corruption, recent research on corruption and anti-corruption within health systems has tended to examine one of these forces (health systems failure, practical norms, social networks, informal rules or economic imperatives) at the expense of exploring how these factors impact on one another. In this paper, drawing on ethnographic data gathered in the Nigerian health system, we examine the intersecting forms and drivers of informality and corruption, to explore how they shift the dynamics of primary health centres and undermine the goals of access and equity set out in policy. The research develops Blundo and de Sardan's commitment to understanding how the health system 'really works' in the PHCs in Enugu, Nigeria (Blundo and De Sardan, 2006). We examine key actors involved in decision-making, the widespread and systematic collection of informal payments and the role of these funds in sustaining services and reinforcing social relations among staff. At the heart of the paper, is the development of an understanding of PHCs as marketplaces: highly local spaces which are governed informally, in which goods and services are bought and sold, proceeds reinvested in goods and/or distributed among staff. The paper develops current work on anti-corruption in public health that prioritises one force as the driver of practice, showing instead how these factors combine to create the necessary conditions out of which care is commodified and a localised healthcare market emerges. We highlight how they cater for the coexisting interests of both

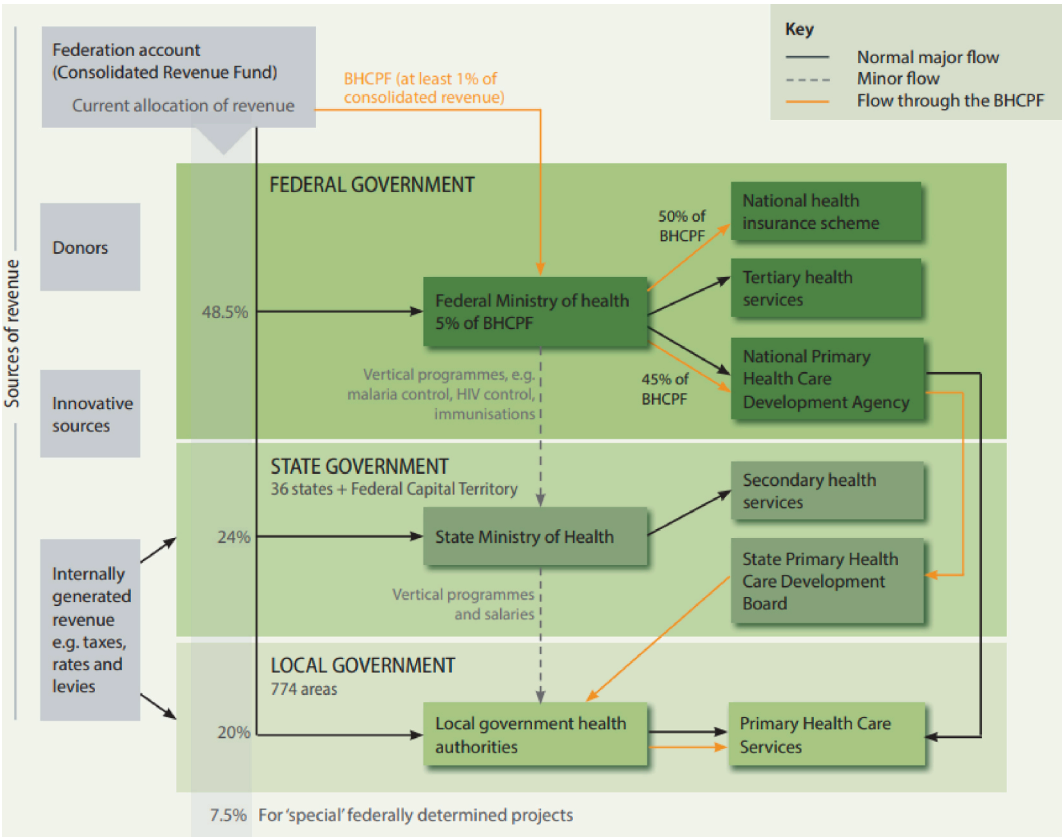

Fig. 1. Flows of funds for health services.

individuals and health facilities themselves and are sustained by a network of actors.

2. Methods

This ethnographic research that we draw on in this study is part of a long-term project on corruption in Nigeria's health system (Agwu et al., 2020; Angell et al., 2023; Obodoechi et al., 2021; Odii et al., 2022; Onwujekwe et al., 2019). Methodologically, the research draws on the Manchester School of anthropology and conceptualises the health system as a social field that forms and re-forms as unequally positioned social actors come together to compete and cooperate over social, material and symbolic health resources (Gluckman, 1961; Turner, 1974). Our approach did not, therefore, necessitate a fixed theoretical framework with key domains of interest which are commonly found in qualitative health research. Instead, field theory drew our attention towards the establishment of networks of actors, the key roles played in these networks by particular individuals, the distribution of power and resources within and between these networks, their relationship to the formal health systems structures that enabled/limited the activities of these groups of actors. Such an approach has previously been used to analyse how formal and informal rules and activities can be combined in health systems and to elucidate the structural position of actors working in the margins of service provision (Hutchinson 2021). As far as we are aware, however, this is the first time that this conceptualisation of the health system has been applied to spaces commonly recognised as part of the formal public health system in order to elucidate the structural conditions which shape care provision in PHCs. The benefit of such an approach is that it enables the development of theory on corruption within health systems and provides space for the development of empirical findings from grounded ethnographic fieldwork (Muir and Gupta, 2018).

The ethnographic research that we draw on for this paper was conducted by a team of four researchers between December 2020 and April 2021. Participant observation took place across four local government areas in Enugu State in a total of six PHC (three urban and three rural) and in the offices of two LGAs headquarters. All of those conducting participant observation were senior researchers, the majority holding PhDs. All were from Nigeria and had undertaken qualitative research on corruption in Enugu prior to the study. The selection of facilities and local governments was guided by previous studies reporting different types of rent-seeking in these spaces (Obodoechi et al., 2021).

As is common practice in anthropology, participant observation began with 2 weeks of unstructured observations when researchers were encouraged to observe a range of events and daily life in the facilities as a means of refining our focus (Kielmann, Cataldo and Seeley 2011). This was followed by six weeks of more structured observations when researchers split their time between either two health centres or one health centre and the local government offices. Brief notes were made during the day if necessary and full fieldnotes were written up at the end of each day. These were shared with the group at the end of each week. For eight weeks of participant observation, we ended each week of data collection with an in-depth team meeting during which time members presented their most important findings of the week, discussed and compared these findings, considered their relationship to the literature and raised unanswered questions and planned activities for the following week. These weekly meetings were attended by two senior staff members from the UK (an anthropologist and a senior health policy and systems researcher) and a senior Nigerian health economist. At the end of eight weeks, the field notes and transcribed data were imported into NVivo software.

The researchers drew upon the coded data to draw up an interview guide with the specific intention of deepening the understanding and interpretation of the findings from the ethnographic fieldwork. Each researcher identified key questions for the in-depth interview guides for service providers and local government personnel, and for FGDs with local residents and placed these into a master list. During two meetings with the entire research team this master list was narrowed into key areas and questions were re-written.

Following the participant observation, using a purposive sampling strategy we selected individuals with experience using and providing health services in PHCs, as well as those in administrative positions who influence the functioning of these facilities as participants for FGDs and in-depth interviews. This included service users, health workers, and managers at the local government level. Overall, we conducted 62 interviews with health workers and managers at local government level and 8 FGDs (6–9 participants in each) with community members. The health workers interviewed included midwives, nurses, and Community Health Extension Workers (CHEWs) and included the Officer-in-Charge (OIC), the administrative head of a PHC, who could belong to any of these staff categories. We interviewed the managers at the local government who were drawn from the four local governments and included the Head of Department of Health (HOD), Local Immunisation Officers (LIO), Head of Personnel Management (HPM), the Supervisory Council for Health and programme heads like Local action committee on AIDS (LACA). The FGDs were conducted across four local government areas with women who have received health services from the PHCs where the observations took place. All the FGD participants were recruited through the assistance of community mobilisers and health facility managers like the OICs.

The interviews and FGDs were recorded, transcribed verbatim by the researchers. Drawing on reflexive thematic analyses method (Braun and Clarke, 2006), two rounds of coding were conducted. In the first round, four researchers independently coded two randomly selected transcripts to develop codes to guide their analyses. The independently generated codes were then jointly reviewed and discussed with the broader project team, leading to refinements. In the second round, the revised codes were applied across the full dataset. Different codes were put together to form a theme after several rounds of sorting, reorganising and renaming. Moreover, the comparison of data between participant observation, in-depth interviews and focus group discussions allowed us to cross-verify findings and strengthened the credibility and validity of our study.

Ethical approval was obtained from INSTITUTION WITH REFERENCE CODE (removed to ensure blind review) and INSTITUTION WITH REFERENCE CODE (removed to ensure blind review). These approvals were necessary because the research involved human participants. Consent to participate was obtained from all health workers, including volunteers at the selected health facilities. Additionally, interview and FGD participants provided a written consent form after reviewing the research objectives, as well as the benefits and risks associated with participation.

3. Findings

1. Health systems failure and funding challenges

Nigeria operates a federal system of government that allocates power to all three tiers, the federal, state and local governments (see Fig. 1). Under the Primary Health Care Under One Roof (PHCUOR) policy, PHCs are organised and controlled by the state and LG authority, and the State PHC agencies are expected to translate policies into action (National Primary Health Care Development Agency, 2018). The state and LG organise the Local Government Health Authority (LGHA) to manage PHCs and report to the State PHC agencies. In principle, the State PHC board is supposed to handle the management of PHC staff as well as funding through a dedicated annual budget from the State Ministry of Health (SMoH).

Most states in Nigeria, including Enugu State, are yet to fully implement the PHCUOR policy. Therefore, the coordination, funding and management of PHCs, including the hiring of and payment of staff, continue to be handled by the LGs and the local Government Civil

Service Commission (LGSC) (Eboreime et al., 2017).

At the LGA level, the body responsible for coordination and oversight function is the Local Government Health Authority (LGHA). The LGHA includes the LG chairman, the supervisory council for health, the HOD/director of PHC, HOD/Director of finance, focal persons for immunisation and disease control and community representatives from the Ward Development Committee. The PHCs in urban areas are busy. As they have a deficit of formal staff most also rely on services of informally employed but trained 'health worker volunteers'. In the rural PHCs where we conducted the research, the services were predominantly delivered by these volunteers rather than staff. PHCs provide a range of services including immunisation services, maternal and child services like delivery and antenatal care. They also provide birth certificates and medical fitness certificates. The services are all interconnected and staffed by health workers (except birth certificate and laboratory) whose shifts are organised by the OICs.

The study found that across all of the PHCs there was poor funding and a lack of commitment to coordinate its activities. Health workers reported that they were not getting enough funding to handle basic needs including critical requirements such as the purchasing of medicines and payment of utility bills. At local government level, we heard reports that funds that should have been allocated to the local government for disbursal to the PHCs had been diverted by actors at the state level. The state and LGs operate a joint account through which they receive their respective share of federal allocation and the state was reported to have reduced the amount released to the LG:

"Every month, when the allocation comes, it will go to where they call JAC (Joint Allocation Committee) where the state committee of finance will go to Abuja and return to verify the allocation. After all, signatories have been done by the financial secretary and accountant general …, then, when the money is paid to the local government, it is the State *of their own volition* that will say okay, 'this is the amount we are going to give to this local government and that local government." (Administrative staff, LGA)

In addition to the lack of funds for medicines and the health centres' infrastructure, both health workers and LG staff reported that staff were rarely recruited and those who were in place were rarely engaged in supervision or training activities. There was also no robust system in place to check the activities of health workers. The LG staff who were supposed to visit health centres to monitor activities had neither funds nor transport to attend in person. As an LG employee explained:

"The problem is that at times, we don't get support to enable us to reach some of the hard-to-reach areas. And because there is no financial support, we do it within ourselves as a team. But if there's support it will encourage you to get to the hard-to-reach areas. But because there's no support everybody will get to reach where they can reach easily and do some supervision. But if there's effective support, with these 6 team members, each person supervising 4 facilities in a month that's at least one health facility per week we will be able to cover all facilities in a quarter." (HOD, LGA)

The lack of funding and supervision means that health centres and the staff that worked in them were effectively abandoned by the local government and state. In the following sections, we explore how with few commodities to distribute, a lack of funds to pay utility bills, a lack of staff and few monitoring visits, PHCs adopted practices beyond those set out in policy. These transformed PHCs into commercial hubs that effectively commodified primary healthcare. We explore how these practices enable facilities to function, by buying medicine and other consumables, compensating 'volunteer health workers' (fully trained health workers who provide services in the health centre but do not have regularised positions with salaries attached) and to strengthen relationships between health workers in each of the facilities.

2. Informal rules in Primary Health Centres

All of the health centres that we observed were busy sites of economic activity, and this took place with and without explicit permission from the LG administrative heads.

In all facilities, there were visits by itinerant traders who carry their goods with them and move from place to place to make sales. These traders who were often resident in the local area visited when they expected patients' numbers to be high, to sell food, adult and children's clothes and herbal creams as a remedy for common skin complaints. Some health centres also allowed individuals to have fixed stalls within the compound, displaying their goods and engaging in petty trade. These included stalls owned by health workers who also sold foodstuff and spent time selling baby clothes and nappies. Volunteer health workers were often allowed to sell food and baby products to patients and carers as a means of supplementing their income. For health workers, being allowed to earn an income like this reduced the challenges of poor remuneration and as for the volunteers, it served as a means to earn transport fees. As one health worker explained.

"Some [volunteer health workers] sell slippers and clothes … I also buy fresh fish and also dried ones. If not that I do this and get transport, I would have quit. What they know me is 'chop and smile'. I do bring buns on immunisation day and the pregnant women buy them before exercise." (volunteer health worker, rural PHC)

The relationships between staff and traders were not always cordial, and one facility had banned traders from setting up stalls within the facility shortly before the research began. The conflict arose over the competition for space between traders and health workers who were selling items to patients. The sale of food, herbal cream, and baby clothes were made in open transactions between staff, the community, and patients and we heard few complaints in the facilities from health workers or patients about this arrangement. In one FGD, however, women seeking care revealed how they found this exploitative. Health workers in one facility made it obligatory for women in labour to purchase consumables for childbirth from them rather than from those outside the facility. Prices in the facility were inflated significantly but women felt that they had to acquiesce, fearing that if they refused, they would risk being denied care.

"One of them [health workers] sells diapers and it was my turn, I was number 9, She collected 500 [for an immunisation], and said that I must buy it [diapers] that [is] her own for 300 naira. So, they are doing it like what Christ said that [is] my father's house they are using it for robbery. They will collect money for the needle and you will also buy diapers. So, the thing is that they will attend to you [only] after you have bought their goods." (FGD female service users, rural)

The process of selling goods within the facility impacted on time given over to caring for patients. Health workers who sold goods were often late to start their duties, spending time setting up their goods and displaying them. There were occasions when they prioritized vending their goods over providing essential care, choosing to attend to the needs of customers over those requiring immediate medical attention.

3.1. Buying and selling medicine and medical commodities

Among services that were overseen by the OIC, we witnessed the buying and selling of diagnostic tests, reagents, medicines, vaccines, consumables associated with vaccines (syringes, cotton wool), and inpatient care associated with childbirth. The laboratories in two of the health centres were run by health workers who were never posted to other sections. These individuals purchased equipment privately when it was not made available by the local government and sold these to patients at prices that they fixed themselves. Payments for tests were often non-negotiable. One researcher (CO), for example, witnessed a woman with a sick baby being refused care when she lacked funds to pay for the child's malaria rapid diagnostic test.

Although according to policy no one should be paying for vaccines for babies and children, charging for these was ubiquitous. At all facilities, payments for vaccines were undertaken in the open (rather than taking place in secret), prices fixed, and the process regimented; we

never saw patients seeking this service without paying, and we never witnessed anyone trying to negotiate the costs. In one urban facility, for example, women who came for vaccines first waited in a queue to see the nurse administering the injection. When it was their turn, they approached the health worker, dropped 500 naira (0.33 USD) into a bucket, handed over their baby's vaccine card, and settled their babies in preparation for the injection. The practice appeared to be well understood by both the health workers and care seekers; we did not witness anyone telling the women the price at the point at which the vaccine was administered and no one asked the health workers if they could pay less.

On one occasion in one health centre, however, researchers witnessed an OIC instructing health workers to reimburse a patient who had been charged for immunisation of her baby. The reason for the reimbursement was unclear, but during a discussion with the woman's husband he commented that the OIC "*believed [he was] a government official.*" (Fieldnote: rural PHC) The implication was that he would know the national policy and that he would either have the power to insist that it was adhered to or the power to report the OIC to a higher body who could reprimand her and her health workers.

The prices demanded for medicines in the facilities were also often non-negotiable. According to official policy, some medicines are allowed to be sold in health facilities but as these medicines are expected to be purchased from the government through the central medical store (CMS) via a drug revolving fund (DRF) (Amibor et al., 2023), prices should have been low and consistent. In practice, the medicine supply by the CMS was irregular – prices fluctuated and medicines were not always available. OICs in all of the facilities that we observed were worried that stockouts of medicines would lead to loss of patients so they made what they described as 'emergency purchases' from private pharmacies nearby. The uncertainties associated with purchases from these private pharmacies made it difficult to control prices. It also made it difficult for researchers and patients to ascertain when charges were made more or less fairly and those that were exploitative.

Over the course of the fieldwork, prices for these medicines were fixed by the OIC and exemptions were rare for those too poor to pay and those without the power to report health workers to officials. In one rural health facility, for example, a visiting policeman who also claimed to be a medical doctor tried to discipline a health worker who wanted to charge a patient for malaria treatment which he believed should be free or sold below the price that she had stated. The health worker providing care was the OIC and despite his position she explained that they had purchased the medicines themselves and therefore had to charge the patients to cover the costs. The policeman accepted her account of the situation. They went on to have a conversation during which time the OIC complained that the health center had not received medicines from the local government for many years, the DRF was dysfunctional, and out-of-pocket payments were essential to provide care. Such pressures also meant that medicines could not always be provided according to the clinical needs of the patient. Instead, quantities were tailored to the patient's capacity to pay for them.

"You might tell the person that their bill is 1000 and maybe the person can only afford 200, you will have to explain to them and administer treatment worth the money they have or you will advise the person to go and source the money for the full treatment." (OIC, urban PHC)

While payments for vaccines, medicines and laboratory services were fixed for most patients, two forms of charging were beyond the control of the OIC and costs varied. These were charges demanded for birth certificates and costs associated with consulting the physician. Distribution of birth certificates is the responsibility of the Nigerian Population Commission (NPC) rather than the Ministry of Health and although the individual sits within the health centre, they are employed by the NPC and not under the OICs leadership. The NPC officials attend to few individuals and have time to spend bargaining with individuals (including refusing to provide services without the payment of high fees). The charges for consultations with the medical doctors were negotiated differently in the different PHCs, although the medical doctors were under the management of the OICs, we never witnessed their involvement in the management of these informal payments.

3. Distribution of funds, social networks and practical norms

With the OIC taking responsibility for the management of the health centres, it was unsurprising that they were also largely in charge of the distribution of resources gathered from the charges within the health facility. During the in-depth interviews, OICs described how they used this money to ensure the health centre was able to continue to function by paying electricity and water bills, employing builders to dig a well, purchasing vehicles and paying security guards. As one, OIC explained:

> "Whatever we get from our service charges, we have to pay for security because if you don't pay them, you know what it is? So, we make sure we pay them, we get close to them … The light also, we are not supposed to be paying for light. There was a time we waited [for the LGA to pay for the electricity bill] and they say we should write; we wrote and our bill was also getting to 3 000 000 [naira] (US $2000). So, at a time I have to go to NEPA and we negotiated and we started paying gradually." (Female OIC, urban PHC)

OICs also used distributed the money to the staff working in the health centres. In each of the facilities, health workers organise themselves around a network and volunteer health workers were socialised into this system. Within this network, they collaborated or reached an informal understanding of how work should be distributed. One of the interviewees reported a fluid working culture where volunteers and full-time staff collaborate to get things done.

"We don't behave [or say] that this person is a volunteer or staff. We cooperate very well, you hardly know if one [person] is a volunteer or full time [staff member] and we do clean up together, we don't leave it for them. Sometimes when I come and the whole place is dirty, I will start mopping the floor and if another person comes, she will join me." (Volunteer health worker, urban PHC)

The OIC typically takes the lead in organizing this network, or at the very least, the network is often structured and coordinated through the OIC, and this includes the distribution of funds among staff (Odii et al., 2024). The dominant principles of distribution within these networks reflected the position of the health workers in the hierarchy of the health centre, delineating between formally employed staff who mostly received more funds than the volunteers. This indicates that while tasks may be fluid, distribution of funds were not and when processes deviated from this, conflict could arise. An illustration is captured in the quote provided by a volunteer health worker:

"That is why we [volunteer health workers] appreciate the efforts of the OIC because she said that the small money the facility gets will be shared equally among both staff and volunteers. The staff were so angry and said other "OICs don't do this [in their health centres]". They [said], "[We] are seniors!" Now, [these staff] … go to the nurse's quarters and sleep, telling the OIC to work with the volunteers." (Volunteer health worker, rural PHC)

Across the health centres, the relationships between members of these distributive networks were reinforced through the participation of health workers and volunteers in various social activities and engagements. These fostered cohesion and camaraderie. In all facilities, there were welfare associations that collected funds to support health workers and volunteer health workers during family emergencies. Health workers and volunteer health workers attended colleagues' wedding ceremonies as a group, adhering to their welfare's financial contribution mandates by contributing money to support newly married couples.

These social networks entailed a form of solidarity that dissuaded any individuals from reporting rent-seeking practices and helps explain their apparent indifference when some colleagues engage in private money-making ventures within the facilities, the proceeds of which do not get shared. Typically, healthcare providers in these settings

refrained from reporting these practices, as one health worker explained, unless the situation becomes unmanageable or directly affects them:

"It is not good; for me, I will not report you until I warn you repeatedly and I will even let you know I am going to report you if you break the rules." (Female health worker, rural PHC)

These distributive networks also reach out beyond individual health centres and could also include health managers from local government who would request a proportion of the funds generated from the facility from the OICs. One of the OICs explained that these managers could use their position in the official hierarchy to enable them demand a portion of the facility's proceeds. As one OIC described:

"The HOD might just call me that she needs money for something. If you have you give her but if you don't you tell her. However, she will not be happy, she will tell you that this place is always busy, and you should find a way to get money. She might not say it but she shows it somehow. If there's anything that comes up at the local government, she will keep it by herself. She might even transfer you on the ground that you are not servicing your table well, or tell you that she will bring somebody that will service the table well. It is very unofficial, it is informal." (Female OIC, urban PHC)

4. Discussion

This paper provides an account of corruption in PHCs in Enugu, Nigeria and describes how poor funding and oversight, strong distributive social networks, shared practical norms and generalised informality shape practice. We argue that together, these forces transform primary health care centres into marketplaces in which healthcare is bought and sold.

Current research tends to privileged a focus on one force at the expense of the others (Hutchinson et al., 2018). The interest in this paper lay in understanding how these elements were interconnected and reinforcing, shaping the health system and the possibilities of equitable access to care. It develops an approach from West Africa among public sector workers that examined how 'practical' norms, substantially different from those set down in formal policy tend to governed practice (Blundo and Sardan 2006).

Our findings resonate with the literature on informal payments, showing how primary health care systems that should provide essential services free of charge or at minimal cost work in practice to commodify health care (Chereches et al. 2013). But, processes of commodification of public services in West Africa are not simply the outcomes of the action of profit maximising individuals (Blundo and Sardan 2006). Forms of corruption at the frontline of public service provision (including education and healthcare) are deeply social, reinforcing relationships between public sector actors and following social ties between these civil servants and their kin-based, political and social networks. Describing these health centres as 'marketplaces' aims to bring alive the understanding that these are dynamic, deeply social spaces in which some prices are fixed, others amenable to bartering, where social ties are reformed and remade as resources are distributed.

Describing primary care centres as marketplaces also underscores the ways in which this paper challenges Blundo and Sardan's proposition that corruption is strikingly similar across sectors and countries in West Africa (in their case, Niger, Benin and Senegal) (Blundo and Sardan 2006). Our methodology reflected interest long held in anthropology in the highly localised nature of practices that emerge through the intertwining of the formal bureaucratic systems with social networks (Lê, 2013). In Enugu, the formal hierarchy of the health system was maintained to the extent that the OIC held responsibility for the functioning of their health centres. As PHCs were effectively abandoned by the local government who provide very little oversight, funding and access to consumables, the leadership position of the OICs in each health centre intensified. OICs took decisions (about personnel, the use of volunteer labour, taking and distributing of many informal charges). While the health centres were of course still part of an overall health system connected to the local government offices, in terms of everyday activities, informal innovations meant that each primary health centre often operated as a distinct unit. These units provided a local marketplace in which a range of commodities and services were provided and sometimes medicines were tailored according to the capacity of the patient to pay for them (a practice associated with medicine sellers in East Africa (Mayora et al., 2018).

In academic and policy texts, corruption in poorer countries is often presented as a force that creates stagnation and backwardness (Muir and Gupta, 2018). The health centres in our study were, however, far from stagnant. Instead, they were busy places in which care was delivered (albeit in a way that undermines equity); goods and services sold; social networks established and cemented and the morality of distribution was debated. At times, informal practice was considered unethical and exploitative (when health workers demand that women purchase their goods at inflated prices) but at others, informal payments were seen as the means through which services could be delivered (when the OIC explained to the policeman that she had to charge for malaria tests). Forms of distribution among health workers could also be contested, their morality judged not in relation to written policy but rather in relation to whether staff members with contracts had more rights over profits in the health centre.

In these cases, corruption is not best understood as an aberration in the system. Instead, following Smith, our paper adds to the literature by demonstrating that the health system emerges as an institution that is formed by the intertwining of bureaucratic functions with those of patronage and the imperative of making and maintaining broad social networks (Smith, 2006). A lack of support from government makes the delivery of health services contingent on existing social relations and practices (gift giving and the distribution of resources as a means of securing social cohesion). Health workers are forming their own social networks that go beyond usual forms of kin- and community-based patronage. Understood in this way, rather than focusing on how to stamp out corruption the question becomes how do we improve ethical, equitable access to health care within the constraints of this system?

We believe that this question is important beyond Enugu state. While the nature of informality and corruption in health centres in other states is likely to be differently manifest, the health system failures identified in this study are widespread across Nigeria. Scholars have consistently shown that the failure of local governments, including their inability to fund PHCs, is linked to the embezzlement and misappropriation of funds generated through taxes, charges as well as allocations from state and federal governments (Kurfi and Danrimi, 2020). This manifests in the control of financial allocations, taxes, and counterpart funding through administrative heads who are primarily party loyalists (Boris, 2015; Diejomaoh and Eboh, 2010). Similarly, it was reported in Oyo, a state in the southwestern part of Nigeria, that the state government has repeatedly violated the law by asserting control over the Local government's internally Generated Revenue (Oladele, 2020). Health worker shortages are connected to irregular recruitment of staff by the local government in Oyo state (Aluko et al., 2019). The use of informal payments to pay volunteer health workers and sort out other necessities in the health facilities has been described elsewhere (Odii et al., 2024; P. A. Ogbozor et al., 2023). Together, these show that if primary health centres stay open and provide services in the face of a constant and profound lack of funding, informal systems will have had to emerge to enable this to be the case. The informal rules for doing so are likely variable but managing poor funding is likely to transform health facilities from places for providing a public service into marketplaces, as health care providers keep the facility functioning and some seek to make profit. This is marked by the integration of complex social processes and activities with economic transactions, blurring the lines between healthcare provision and commercial exchanges (Bestor, 2001). This shift challenges the traditional understanding of these centres as focal points for the fair distribution of health services.

Gaitonde identifies these forms of corruption as "survival corruption" which may enable access to health services for some in the short term but often make it difficult for marginalized groups to gain care (Gaitonde, 2019). Certainly some partial or no exemptions can be granted but this was rare in our study. Similarly, in Ghana, the emphasis on individual health centres sustaining their operations, survival corruption rarely leads to equitable distribution of services (Agyepong and Nagai, 2011). As in other studies, informal practices or survival corruption that occurred in response to the constraints of the health system mixed with practices that were more straightforwardly exploitative (Mirhosseini and Fattahi, 2010). Demands that patients purchase products at inflated prices from health workers appear to be possible not because they are acceptable and part of a set of norms shared between staff and patients but rather because the health workers were in powerful positions and patients feared that they would be denied services if they did not acquiesce. High levels of cooperation among staff, underpinned by enduring relationships, which were both friendly and professional in nature (Moore et al., 2015), meant that other staff would be unlikely to make complaints against them. Our research suggests that terms such as survival corruption are important to flag informality that emerges as a response to a lack of government investment but that we need to explore how this interconnects with and enables exploitation through the abuse of trusted power for private gain.

5. Conclusions and implications

In this paper, the Nigerian health system has emerged as one formed of a combination of bureaucratic processes, ties of patronage and social networks formed among professional groups. Corruption does not appear to be an aberration in the system, the consequence of an amoral individual seeking to maximise their own profit or benefits. Instead, it is a central part or logic of the way in which services are provided and received. In a context in which commodities are rarely forthcoming from the state, healthcare facilities are better conceptualised as 'marketplaces' overseen by powerful OICs who organise the distribution of resources. Overall, Corrupt practices become routine, a culture of exchange prevails, but the poorest and least powerful of who will likely fall victim to the high cost of care. But, these forces change the nature of the health system as it is described in policy and undermine processes designed to enable a more just, equitable system to emerge.

Recent plans, such as the Basic Health Care Provision Fund (BHCPF), may address some health facility needs (National Primary Care Development Agency et al., 2020). Additionally, there is the prevailing issue of challenges in DRF, an issue extensively documented in the literature (Okeke et al., 2021). But even with investments in these areas, understanding the health system as an intertwining of these bureaucratic and patronage systems makes it unlikely that these will substantially change the functioning of the system. How then do we re-work anti-corruption to manage these processes? Large-scale, sector wide anti-corruption programmes seem unlikely to be effective. Instead, there needs to be greater accountability from the State PHC board in overseeing how funds are allocated, disbursed, and spent within PHCs as well as tackle challenges with the supply of basic drugs and medical supplies. Beyond this, new bureaucratic processes need to emerge that safeguard the poorest and ensure that they are able to access care in the current system.

CRediT authorship contribution statement

Aloysius Odii: Writing – review & editing, Writing – original draft, Methodology, Formal analysis, Data curation, Conceptualization. **Eleanor Hutchinson:** Writing – review & editing, Writing – original draft, Supervision, Methodology, Funding acquisition, Conceptualization. **Obinna Onwujekwe:** Writing – review & editing, Supervision, Funding acquisition, Formal analysis. **Prince Agwu:** Writing – review & editing, Methodology, Formal analysis, Data curation. **Tochukwu C. Orjiakor:** Writing – review & editing, Methodology, Formal analysis, Data curation. **Pamela Ogbozor:** Writing – review & editing, Methodology, Formal analysis, Data curation. **Martin McKee:** Writing – review & editing, Supervision, Methodology, Funding acquisition, Formal analysis. **Dina Balabanova:** Writing – review & editing, Supervision, Methodology, Funding acquisition, Formal analysis, Conceptualization.

Ethics statement

Ethical approval was obtained from Health Research Ethics Committee of London School of Hygiene & Tropical Medicine (LSHTM ref: 14 540–1) and University of Nigeria Teaching Hospital Ituku-Ozalla, Nigeria (NHREC/05/01/2008B-FWA00002458-IRB00002323). These approvals were necessary because the research involved human participants. Consent to participate was obtained from all health workers, including volunteers at the selected health facilities. Additionally, interview and FGD participants provided written consent form after reviewing the research objectives, as well as the benefits and risks associated with participation.

Declaration of competing interest

None.

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Data availability

The authors do not have permission to share data.

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